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Bristol-Myers Squibb Company, Q1 2026 Earnings Call, Apr 30, 2026

Apr 30, 2026

Transcript Summary

Company Performance

The company reported a solid performance in the first quarter of 2026, with total revenue up 1% year-over-year, reaching approximately \$11.5 billion. The growth portfolio saw a significant increase of 9%, contributing to the overall revenue. Key products like Eliquis, which generated approximately \$4.1 billion in revenue (a 13% increase), performed well, despite facing challenges from price reductions. The company reaffirmed its financial guidance for the full year, indicating confidence in tracking towards the upper end of established revenue and earnings per share (EPS) guidance ranges. Overall, the company is positioning itself as a fast-growing player in the pharmaceutical sector.

Growth Plans

The company has a clear focus on expanding its growth portfolio by advancing its pipeline of innovative products. It aims to deliver more than 10 new medicines and 30 life cycle management opportunities by the end of the decade. Recently, the company made progress in its regulatory and clinical milestones, including the filing for iberdomide for multiple myeloma with Breakthrough Therapy Designation and positive Phase III interim data for mezigdomide. Additionally, the approval of Sotyktu for psoriatic arthritis and Opdivo for new indications highlights the company's strategy to broaden its therapeutic areas. The company plans to continue evaluating business development opportunities in its core therapeutic areas while returning cash to shareholders through dividends.

Key Initiatives

Central to the company's strategy is a disciplined approach to research and development (R&D). Investments are being made in enhancing R&D productivity through talent upgrades, streamlined decision-making, and improved management of clinical activities. The company is also leveraging AI tools and laboratory automation to enhance the quality of its pipeline. A strategic productivity initiative is expected to deliver \$2 billion in cost savings by the end of 2027, allowing for further investments in growth-oriented opportunities. The focus remains on developing treatments for life-threatening diseases while maintaining a balanced capital allocation strategy that prioritizes shareholder returns.

Risks

The company faces financial risks such as declining gross margins, which decreased by 280 basis points to 70.3% in the first quarter, primarily due to product mix. Additionally, the impact of generic entries on

legacy products poses a challenge to revenue growth. The company has acknowledged a temporary inventory build due to price reductions and anticipates a reversal in the subsequent quarter. To mitigate these risks, the company is focusing on strategic investments, enhancing its product portfolio, and continuing to execute on its operational initiatives to maintain flexibility in cash flows. The management remains committed to closely monitoring performance and adjusting strategies as needed to navigate potential challenges ahead.

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Original Document

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Presentation

Operator

Welcome to the Bristol-Myers Squibb First Quarter 2026 Earnings Conference Call. [Operator Instructions] Please note this event is being recorded. I would now like to turn the conference over to Chuck Triano, Senior Vice President and Head of Investor Relations. Please go ahead.

Charles Triano

Thank you, and good morning, everyone. We appreciate you joining our first quarter 2026 earnings call. With me this morning with prepared remarks are Chris Boerner, our Board Chair and Chief Executive Officer; and David Elkins, our Chief Financial Officer. Also participating in today's call is Adam Lenkowsky, our Chief Commercialization Officer; and Cristian Massacesi, our Chief Medical Officer and Head of Global Drug Development. Earlier this morning, we posted our quarterly slide presentation to bms.com that you can use to follow along with Chris and David's remarks. Before we get started, I'll remind everybody that during this call, we will make statements about the company's future plans and prospects that constitute forward-looking statements. Actual results may differ materially from those indicated by those forward-looking statements as a result of various important factors, including those discussed in the company's SEC filings. These forward-looking statements represent our estimates as of today and should not be relied upon as representing our estimates as of any future date, and we specifically disclaim any obligation to update forward-looking statements even if our estimates change. We'll also focus our comments on our non-GAAP financial measures, which are adjusted to exclude certain specified items. Reconciliation of certain non-GAAP financial measures to the most comparable GAAP measures are available at bms.com. Finally, unless otherwise stated, all comparisons are made from the same period in 2025, and sales growth rates will be discussed on an underlying basis, which excludes the impact of foreign exchange. All references to our P&L are on a non-GAAP basis. And with that, I'll hand it over to Chris.

Christopher Boerner

Thanks, Chuck. Welcome, and thank you for joining our first quarter earnings call. We delivered a solid Q1 and continue to improve our say-do ratio with disciplined execution across the business as we continue to best position the company for long-term sustainable growth. Our strategy remains grounded in 3 priorities: focusing R&D on life-threatening diseases, driving strong execution across the organization to build momentum in our growth portfolio and maintaining disciplined shareholder-friendly capital allocation. We saw progress across all 3 in the quarter. Let me start by highlighting our performance on Slide 4. We started off the year with solid results across our key marketed products. In the quarter, growth portfolio sales were up 9% year-over-year with contributions from a broad range of assets, including Reblozyl, Breyanzi, Camzyos, Opdualag, Qvantig and Cobenfy. These are differentiated, durable assets that treat serious diseases and remain early in their life cycles, and they continue to strengthen our foundation for long-term growth. Overall, our growth portfolio performed in line with our expectations for this quarter. Outside of the growth portfolio, Eliquis performed well and grew in line with the range we provided on our Q4 call. David will provide more details on the financials shortly. Turning to our recent regulatory and

clinical milestones. In Q1, we made progress advancing our broad and diversified pipeline. Regarding our CELMoDs, iberdomide and mezigdomide, our iberdomide filing for relapsed or refractory multiple myeloma was accepted by the FDA with Breakthrough Therapy Designation and priority review with a PDUFA date of August 17. This is an important step for our protein degradation platform, potentially enabling us to bring the first CELMoD to market. For mezigdomide, we reported positive Phase III interim data from the SUCCESSOR-2 study, demonstrating a meaningful improvement in progression-free survival in patients with relapsed or refractory multiple myeloma. This marks the second positive pivotal readout from our oral CELMoD program and further strengthens our conviction in the platform. We will also present the full data at ASCO and are actively planning regulatory submissions based upon the data. For our ADC, Iza-bren, we shared positive Phase III interim top line results in patients with previously treated triple-negative breast cancer based on a study conducted in China. We will present these exciting data along with the positive Phase III China study results for Iza-bren in previously treated esophageal squamous cell carcinoma at ASCO. At the same time, we continue to broaden the reach of our in-market portfolio through life cycle expansion. We received approvals for Sotyktu in psoriatic arthritis and Opdivo for 2 new classical Hodgkin Lymphoma indications. We also reported positive Phase IV switch data for Cobenfy, positive Phase III data for Camzyos in adolescents with obstructive HCM and positive Phase II data for Reblozyl in alpha thalassemia. Stepping back, these updates reflect the diversity and breadth of our pipeline, both in terms of therapeutic areas and modalities, as well as continued execution across the business. Moving to Slide 5. As we've said, the latter part of 2026 is shaping up to include an increasing cadence of pivotal readouts that are expected to further define and derisk our long-term growth profile. Among the Phase III readouts expected late in the year are milvexian in atrial fibrillation and secondary stroke prevention, Cobenfy in Alzheimer's psychosis, admilparant in IPF and iberdomide PFS data. We anticipate these readouts will help us further diversify and broaden our portfolio and are part of our efforts to deliver more than 10 new medicines and 30 meaningful life cycle management opportunities by the end of the decade. Turning to Slide 6. Central to delivering on these opportunities and enabling sustained long-term growth are our efforts to drive top-tier R&D productivity. In our development organization, we continue to improve execution across drug development by upgrading talent, streamlining decision-making and instituting tighter management of core clinical activities. We are also focused on enhancing the quality and depth of our early to mid-stage pipeline. Underpinning these efforts are investments we are making in core R&D infrastructure, including broadening the use of AI tools together with laboratory automation and people trained in the right ways of working. In research and early development, target selection and molecule design can have an outsized impact on long-term value. We have set a target to reach lead molecule identification approximately 50% faster while applying greater rigor so that only the most differentiated molecules advance. In late development, we're using AI to streamline clinical operations, compress development time lines and enhance quality oversight. Over time, we expect these efforts to deliver a 30% reduction in cycle times versus just a few years ago. Among others, we have ongoing partnerships with Faro, enabling us to design trials more efficiently and Evinova's Cost Optimizer tool. These ongoing efforts across R&D are top priorities for 2026. The organization's continued focus on financial discipline enables us to make these and other important investments. We remain on track to deliver the remainder of our \$2 billion in cost savings from our strategic productivity initiative by the end of 2027. With respect to capital

allocation, business development remains an important focus. As always, we will continue to index on opportunities where we add strategic value and where we can deliver attractive returns. As our post-LOE growth profile becomes clearer, we'll naturally place greater emphasis on expanding our early and mid-stage portfolio to support growth into the 2030s. In summary, based on our performance, we see the business currently tracking towards the upper end of our guidance ranges. Looking forward, we have continued momentum in our growth portfolio, broad potential in our pipeline and the ability to invest in our business while becoming more focused and efficient in how we operate. With that, I'll turn it over to David.

David Elkins

Thank you, Chris, and good morning, everyone. Our performance in 2026 is off to a strong start as highlighted by our first quarter results. We delivered solid R&D, commercial and financial performance while continuing to manage our cost structure. Our persistent focus on execution has further strengthened our foundation as we position the company for long-term sustainable growth. I will begin with a review of our first quarter results and then discuss our financial outlook for the remainder of the year. Starting with Slide 8. Total revenue in the first quarter was up 1% year-over-year at approximately \$11.5 billion. Our growth portfolio continued to perform well with global revenue increasing 9% to \$6.2 billion. As Chris mentioned, several products that are still early in their life cycles are driving growth as we intentionally expand our business across a wider range of key assets. Within the legacy portfolio, we saw solid growth from Eliquis, which was offset by the continued impact of increased generic entry across several other brands. All in, we are very pleased with our results in the quarter as we build upon our objective to reshape and redefine BMS as one of the fastest-growing pharmaceutical companies into the next decade. Turning to product performance on Slide 9, starting with oncology. Opdivo revenue decreased 8% to approximately \$2.1 billion, with most of this decline coming from the U.S. This was primarily driven by an Opdivo inventory drawdown at the wholesaler level, where inventories are at the low end of the typical range. We continue to monitor whether these levels will normalize over the balance of the year. In addition, we saw continued conversion to Qvantig, where the launch continues to progress well with revenues of \$163 million. With Opdualag, we delivered another quarter of strong double-digit growth driven by demand globally, where it remains a standard of care in first-line melanoma. Turning to Slide 10. Reblozyl delivered 15% growth with performance continuing to reflect solid uptake across first- and second-line MDS-associated anemia. In cell therapy, Breyanzi's first quarter growth of 53% reflects its best-in-class profile and continued strong demand across its approved indications in both the U.S. and international markets. We remain encouraged by Breyanzi's continued momentum and growth prospects. Moving to cardiovascular and immunology on Slide 11. Eliquis revenue was approximately \$4.1 billion in the quarter, an increase of 13%. We continued to see strong demand. And given our U.S. price reduction that took effect at the beginning of the year, we also saw some wholesaler inventory build in the first quarter. We anticipate this build to reverse in the second quarter. Turning to Camzyos. Revenue in the first quarter nearly doubled to \$314 million, benefiting from continued demand growth globally. Now moving to immunology. Global revenue of Sotyktu grew 20%. The recent approval in psoriatic arthritis represents a continued presence in rheumatology while we await our Phase III readouts in Lupus and Sjogren's disease. I will wrap up our product performance on Slide 12 with neuroscience, where Cobenfy revenue in the first quarter was \$56 million, representing

continued steady growth. Now let's move to the P&L on Slide 13. As expected, gross margin declined 280 basis points in the first quarter to 70.3%, which was primarily driven by product mix. Excluding in-process R&D, operating expenses for the first quarter were \$3.9 billion, slightly above the same period last year. As compared to a year ago, the incremental investment related to Pumitamig, Qvantig and Cobenfy was largely offset by savings from our strategic productivity initiative. This continues to provide additional flexibility to invest behind these growth-oriented opportunities. Our effective tax rate in the quarter was 18.3%, reflecting jurisdictional earnings mix. Overall, diluted earnings per share was \$1.58 for the quarter, which includes a net charge of \$0.03 a share related to in-process R&D and licensing income. Turning to the balance sheet and capital allocation highlights on Slide 14. Our financial position remained strong with approximately \$11 billion in cash equivalents and marketable securities as of March 31. In the first quarter, we generated approximately \$1.1 billion in operating cash flow. This quarter's cash flow reflects roughly \$1.2 billion and lower net cash collections due to Eliquis list price reductions. We expect this to be more than offset later in the year through lower rebate payments. In terms of capital allocation, we continue to take strategic and a balanced approach to deploying our strong cash flows. Business development remains a priority, and we are regularly evaluating opportunities in the therapeutic areas we know best while continuing to return cash to shareholders through our commitment to the dividend. Now moving to guidance on Slide 15. We are reaffirming our financial guidance for the full year of 2026. Based upon the first quarter results and our current projections, we see our financial performance tracking towards the upper end of our established revenue and EPS guidance ranges. We will continue to provide updates as the year progresses. In closing, our strong performance in the quarter reinforces our confidence in our ability to deliver long-term value for our patients and shareholders. And to reiterate Chris' comment, our strategy remains grounded in 3 priorities: focusing R&D on life-threatening diseases, driving strong execution across the organization to build momentum in our growth portfolio and maintaining disciplined shareholder-friendly capital allocation. And with that, I'll now turn the call back over to Chuck for Q&A.

Operator

[Operator Instructions] The first question today comes from Asad Haider with Goldman Sachs.

Asad Haider

Great. Just maybe just to open, just given how consequential the clinical readouts at the end of this year are going to be for the company, Cristian, just starting with you, can you just level set us on your confidence in the key programs, specifically for milvexian AF and the Cobenfy ADEPT trials? Just any quantitative bars for success? And then related for Chris, how do the timing of these readouts impact the company's BD strategy as you think about the different outcomes that could unfold with the results of each of these readouts? And where do you see opportunity as you scan the landscape ahead of these readouts? And any framing on potential size of the BD aperture would be helpful.

Christopher Boerner

Thank you, Asad, for the question. Cristian, do you want to start, and then I'll take the BD question.

Cristian Massacesi

Thank you, Asad, for the question. Let me start with the fact that we have a very data-rich 2026 and even probably more '27. And let me start with what we already achieved because in [indiscernible], I think we had a positive MRD with iberdomide in EXCALIBER in '25, and we are expecting the PFS later this year. You know as Chris mentioned in his opening remarks, the PDUFA date for this filing is August 17. We got breakthrough designation, priority review. So it's progressing at pace. We achieved mezigdomide this year with SUCCESSOR-2 oral presentation at ASCO, really looking forward to show you guys the data. In [indiscernible], we will have a readout also with our Arlo-cel that is a GPRC5D CAR-T that is happening later this year. Moving cardiovascular, milvexian, as you was mentioning, are important readout. Both AFib and SSP are -- continue to be expected by the end of the year. We have -- we are recruiting the events. It's an event-driven. Of course, we [remain] blinded. We are recruiting the events as planned. And we have the DMC that regularly is reviewing the data. And even in the most recent meeting, they're recommending to continue the studies as planned. So BMS will be the only Factor XI company with Factor XI inhibitor in AFib. Of course, with the presence also in SSP, this can allow us to continue to lead in the thrombotic space. Confidence remains absolutely unchanged for milvexian in this space. In neuroscience, we continue to expect the ADP studies, ADEPT 1, 2 and 4 by -- readouts by the end of the year. This is based on very -- we are managing the studies and moving the studies more or less at the same -- with the same time lines. So they are lining up quite nicely. We need 2 studies probably for an approval. This is the base case, but -- for a filing, but we will see how the evolution in this space is happening. The confidence remain unchanged also for Cobenfy. Study designs, trial conduction is now completely under control. And of course, the reason to believe in Cobenfy in this space are very clear. Prior data, the data that we have seen in schizophrenia and of course, the open label in ADEPT-1, where patients before being randomized received Cobenfy for 12 weeks, give us confidence in potentially bringing this drug in patients with Alzheimer's disease and psychosis. Last but not least, to me, critical readout this year is admilparant in immunology, in IPF and PPF. This is a novel mechanism, LPA1. This is an inhibitor that can bring a novel mechanism to patients with this very, very difficult to treat disease. It's a first-in-class asset with a very differentiated profile, not only on the efficacy side, but also on the safety side. IPF is guided by the end of the year, PPF will be just a few months later, probably beginning '27. Very solid Phase II data. We -- I'm very pleased on the execution of the Phase III programs. So this is another important therapeutic option. As Chris mentioned, there is much more this year, next year. I have to say, it's an exciting time to be BMS.

Christopher Boerner

Thanks, Cristian. And then, Asad, just on your BD question. Look, BD continues to be a top capital allocation priority for us. It's not impacted by the end-of-year readouts. We have a very strong late-stage pipeline. We certainly don't need to chase deals. But as we've said consistently, if there are opportunities that make sense for us to enhance near-term growth, we have the financial flexibility to be in the mix. At the same time, we're building for the long term, and we're going to continue to add to our early and mid-stage pipeline as well. And of course, given the size of those deals, we can certainly do both. Irrespective of phase of development, though, the opportunities we're looking for in areas we know well scientifically, where we can add clinical and commercial value and, ultimately, we can deliver value to patients and to our

shareholders. We're size agnostic, as we've been, and we certainly have the financial horsepower to go after multiple sized deals.

Operator

The next question comes from Geoff Meacham with Citi.

Geoffrey Meacham

Cristian, on pumitamig, I wanted to check on the cadence of data in, say, the next 6 to 12 months. And would you wait for more mature data on PFS before you really expand the number of trials? Or are you ready to go right now? And then real quick on -- for Adam, just to talk through the Qvantig dynamics versus Opdivo and where you're seeing the biggest demand?

Christopher Boerner

Thanks, Geoff. Cristian and Adam.

Cristian Massacesi

Thank you, Geoff, for the question on pumitamig. Let me start with ASCO. ASCO will be an important meeting this year for PD-1/VEGF inhibitors. There are some competitor data in plenary that, of course, increase the confidence in the class. And we are presenting as an oral, our Phase II data in non-small cell lung cancer in frontline as a global data set after the China data we presented previously in other indications. In a few days, the abstract will be released. Our strategy with pumitamig is replace and expand. We want to replace PD-1, PD-L1 inhibitors, and we want to expand beyond them. We announced and we delivered 7 pivotal studies across indications. All of them are ongoing and all of them are recruiting actively. What is very important in my view is also what we are doing beyond the first wave of trials. The confidence, in my view, is becoming more and more tangible in terms of level of activity, a combinability that you have with PD-1/VEGF, PD-L1/VEGF inhibitors, bispecifics. And in my view, this is potentially translatable across indications. Now the next step, at least in our strategy, is novel, novel combinations. And BMS has a very rich oncology portfolio. BioNTech has also an important portfolio of oncology assets. And what we are doing, we are combining now -- we started the combination of pumitamig with these other drugs that represent an enabler for novel regimens, also using some combination with external partners. On our internal side, we started the combination with our Iza-bren ADC, which is an EGFR HER3 ADC. You will see data, as Chris was mentioning, at ASCO in esophageal, in triple-negative. Is a very active ADC. And I think with pumitamig can represent a very powerful regimen. We started a combination with our PRMT5 inhibitor, our Navlimetostat, a very [indiscernible] drug. So in summary, I have to say that the partnership with BioNTech is moving very well because we are progressing the development of this drug with speed. We think we are very well positioned to make this drug as a potential new backbone in immuno-oncology and generating the next regimens very powerful cancer patients across indications.

Christopher Boerner

Adam?

Adam Lenkowsky

Yes. Geoff, thanks for the question. As it relates to Qvantig, we're pleased with the Qvantig launch performance. Our teams are executing well. And we're seeing use across multiple tumor types. We're seeing uptake in our monotherapy indications as well as in combination treatment, so in RCC, in gastric cancer and in melanoma. We're continuing to hear positive feedback from community oncologists that Qvantig improves practice efficiency with a 3-minute in-office injection and that patients prefer Qvantig when offered the opportunity versus the IV formulation. So we've now delivered over 10% conversion from IV to Qvantig in the U.S. in just over a year on the market, and we're tracking well against our expectations. And we remain confident in our expectation that physicians will convert approximately 30% to 40% of IV business in the next 2 years.

Operator

The next question comes from Alexandria Hammond with Wolfe Research.

Alexandria Hammond

On milvexian, given the size and breadth of the Librexia program, it seems like there's probably a rich set of outcomes between the clean win and a miss. Can you help us think through how you'd approach a subgroup analysis, particularly for patients where the risk/reward calculus might be more favorable for Factor XI? And to the extent that the top line doesn't meet that primary endpoint cleanly, is there a path where a specific patient population still supports a meaningful commercial opportunity?

Christopher Boerner

I'll have Cristian take that, and then, Adam, you can add any color commentary as you need to.

Cristian Massacesi

I mean, Alex, let me start that -- to tell that we continue to be on track by the end of the year with both AFib and SSP. These trials are event driven. We remain blinded. As said, the DMC continues the oversight to both studies. We are at a point in which we -- this give us confidence that we are progressing on the right way with both efficacy and safety. Everything is continuing as planned. In AFib, the study will test noninferiority versus apixaban. And then we will have superiority testing for bleedings. Based on what we have seen in other trials recently and based on the expectation and how we size and power the study, I think we are very much on track with both the endpoints to show noninferiority and superiority in bleedings.

Christopher Boerner

Adam?

Adam Lenkowsky

Yes. Alex, as it relates to commercial opportunity, milvexian represents a significant opportunity commercially. There is a need for a medicine with low bleeding risk, both in AFib and in SSP. And we think

there's a significant advantage to having both indications. We would expect broad adoption. As we talked about previously, fear of bleeding continues to be the main reason why physicians hold back from utilizing Factor Xa in more patients. And despite the highly effective dose like Eliquis, roughly 40% of patients who should be anticoagulated are either untreated/underdosed or they discontinued treatment, and that's driven largely by concerns around bleeding risk. So this leaves a meaningful unmet need across a substantial number of patients. And as a reminder, this study in AFib was designed to demonstrate a superior bleeding profile compared to Eliquis with comparable efficacy. So we believe this profile is going to drive significant demand and will be important for both patients and providers. And as Cristian said, we're looking forward to the data readout at the end of this year, and we think this has true blockbuster potential.

Operator

The next question comes from Chris Schott with JPMorgan.

Christopher Schott

Just 2 for me. Maybe first, can you just talk about Camzyos dynamics post your competitor approval? Just what are you seeing in the market? And just how you're thinking about that evolving? And the second one for me was coming back to the CELMoDs that with kind of some of this initial clinical data has read out. Can you just elaborate a little bit more the role in the market you see for those products based on these initial data sets? And how much of the -- your excitement here is based more on the future readouts versus what we're seeing initially here?

Christopher Boerner

Thanks for the question, Chris. Adam, I think you can take both.

Adam Lenkowsky

Yes. Thanks, Chris. So Camzyos continues to have very good momentum. Again, our commercial teams are executing very well in the field. We are seeing continued strong new patient starts, coupled with high persistency rates. Physician and patient feedback are very favorable and physicians consistently cite the significant and rapid improvement in symptoms, and we're seeing very low drop-off rates. In fact, we are approaching 25,000 patients now prescribed Camzyos in the U.S. with thousands more prescribed internationally. As far as what we're seeing in the field, we've been planning for competition for some time. HCPs continue to reinforce that they see little differentiation. It's still early. Some physicians have started 1 or 2 patients on the competition, and they are still operationalizing their own REMS program. But we also hear consistently that the Camzyos REMS process is very clear and the infrastructure and workflow that have been established now for 4 years are very clear. And thus, PLs have shared they'll use the Camzyos dosing and echo regimen at 4 weeks. So we see this as a positive. Why is that? Because roughly 90% of patients on Camzyos are on the 5-milligram starting dose. And it's simply 1 dose titration to 10 milligrams. So it's 5 mg or 10 mg representing 90% of our business, which is effective for the majority of patients. Camzyos patients feel better in a matter of weeks, where we see from the competition requires multiple

titration steps to reach an effective dose. And so our teams were well prepared for the launch of Aficamten, and we remain confident that we'll be the leader in the space longer term. So as it relates to iberdomide, I think we're really excited about the launch of iber. And I know that Chris and Cristian have talked about this. And so I think there are a few areas that I could point to. Number one, with iberdomide, this is an area that we know very, very well in multiple myeloma. We see that for multiple myeloma, it's a highly competitive, it's a fragmented market, but there remains a need for more effective and safe options that can address the majority of patients, particularly those patients who are treated in the community setting. And that's 70% to 80% of patients. What we hear from physicians, they're excited about oral, low-burdensome regimens that can find a better experience for their patients. And we're confident that iberdomide will provide a balance of high potency, manageable toxicity combinability with daratumumab with the convenience of an oral treatment that amplifies the efficacy of IMiD-based regimens. So our goal is to make both iber and mezi foundational in multiple myeloma, replacing Revlimid and Pomalyst in second line over time in the community, longer term serving as partners for T-cell redirecting therapies and cell therapy. So we know the work that we need to do to establish both iber and mezi in the market, and we are very excited to bring both of these important medicines to patients because we believe this is a real attractive commercial opportunity.

Cristian Massacesi

And, Adam, allow me to step in. Chris, I want to use the opportunity you mentioned in the question on Camzyos because I received -- we received often the question about the nonobstructive HCM plans. I think, first of all, let me start that the level of benefit expected in nonobstructive is different than in obstructive because the heterogeneity of patients and disease is much higher. That said, we have learned a lot from ODYSSEY. We know better what the patients and which diseases they are affected by that can benefit most from a myosin inhibitor like Camzyos. So I want to announce that we are planning now to run a new, more focused study in nonobstructive HCM. And then, of course, the detail of it will be highlighted in ClinicalTrials.gov, while we are progressing this work.

Operator

The next question comes from Evan Seigerman with BMO Capital Markets.

Evan Seigerman

And another one for Cristian. So you really inherited the design and kind of -- of the milvexian trials. Can you walk me through the aspects of the trial design, patient selection that increased your confidence in a potentially successful readout later this year?

Christopher Boerner

Cristian?

Cristian Massacesi

Thank you, Evan, for the question. I think your question is referring to AFib specifically, I imagine, because SSP [indiscernible]. So in AFib, let me tell you that, first of all, the data are solidly based on a Phase II study that we run in total knee replacement that is a very good surrogate for antithrombotic drugs. We learned a lot from Eliquis in that space, and we know the predictivity value of that kind of population for antithrombotic agents. I think the very elegant, refined work that has been done with milvexian was in selecting the dose for AFib. That was -- in that trial, we tested multiple dose levels. And ultimately, we landed with 100 milligram twice a day that is a much higher dose, for instance, that we are using in SSP. But it's a dose that gave us the confidence based on the work that has been done, the modeling and the work that we were able to bring in, in the Phase II part, give us confidence to have at least the same level of efficacy than what we expected with apixaban, preserving, of course, the bleeding value. Going specifically to your question, the design of the study is to be able to show a noninferiority versus apixaban in a head-to-head comparison in a very well-sized study. We recruited 20,500 patients. So very well powered for noninferiority margins. We disclosed these margins. Of course, they go from 0.8 to 1.3. And we will then, when a noninferiority will be met, test superiority for bleedings. We test the split also for major bleeding and nonmajor clinical relevant bleedings because I want to link what Adam was telling. This is a major problem in the clinical setting. And being able to show that the drug provide benefit in terms of decreasing the bleedings will be very important in the marketplace. So as I said, the study is fully powered, well designed and pick, in my view, the right dose to being able to show what the study predefined criteria should meet.

Christopher Boerner

Thanks Cristian. Thanks, Evan, for the question.

Operator

The next question comes from Michael Yee with UBS.

Michael Yee

Following up on the design of the milvexian study. If you take a look at the recent AFib results, you can see that the stroke rates are quite historically a lot lower than they were back in the original days of Eliquis. So just thinking about whether you've taken that into consideration and to what extent you think that impacts the study design and whether you think that there's any chance that the study results will ultimately end up in 2027, which could end up being more positive for you than in 2026?

Christopher Boerner

Cristian?

Cristian Massacesi

Okay. Thank you, Michael, for the question. I don't -- we cannot disclose baseline characteristics and which kind of events we are recruiting. By the way, we remain blinded, of course, to the study. What I can tell you, again, is that the AFib study, Librexia study, has the right sample size and of course, has a predefined

number of events. We are recruiting the events as expected. And the events predefined number is for efficacy and of course, for safety. So we are on track by year-end. It's event-driven. We are in April. We will see later in the year if this event rate will change or not. For the moment, we are on track for a year-end readout.

Operator

The next question comes from Akash Tewari with Jefferies.

Akash Tewari

So for your Cobenfy Alzheimer's psychosis studies, you have several trials, but I know ADEPT-4 requires patients to have a confirmed Alzheimer's diagnosis using both imaging and blood-based biomarkers. Can you talk about why you added that criteria for the study? And was it based on any issues you saw with the ADEPT-2 trial conduct? And then just on CELMoDs, can you talk about your confidence on the SUCCESSOR-1 study, which goes head-to-head against Pomalyst showing a clinically meaningful effect size based on the results you saw in SUCCESSOR-2?

Christopher Boerner

Cristian?

Cristian Massacesi

Thank you, Akash. The Cobenfy ADEPT-4 decision to go into a biomarker selected population was based in a try to decrease the heterogeneity in the patient population. As you know, ADEPT-2 was a study that was already ongoing in the moment we acquired Cobenfy. And we wanted to have a more predefined patient population to be recruited into an Alzheimer's disease psychosis setting. This is why we took this approach of biomarker positivity, which can be done through plasma or radiologically. And I think, of course, this is increase the confidence that the right patients are treated in the trial, even if increase a little bit the operational challenges because, of course, we need a predefined number of patients, biomarker positive, to run. So the screening failures are a little bit higher. This is -- doesn't take out any confidence of the potential benefit that you can see also maybe in a trial like ADEPT-2, where we do not have a biomarker positive study because, ultimately, you treat symptoms, but give more confidence that you have Alzheimer's patients. This is the main reason. We are not the only one doing this -- taking this approach in the space. Going back to your second question on CELMoD, I think SUCCESSOR-2 was a very good news, not only because [it] -- and as you mentioned, SUCCESSOR-2 is an add-on study on top of Kd, but because it came earlier than expected, we hit an interim PFS. You will see the data at ASCO, but you already know that when you hit an interim PFS, it means that you hit the bar that is higher according to what the study was designed for. This is answering your question on SUCCESSOR-1. We believe mezigdomide is a very potent CELMoD. It's more potent than iberdomide, for instance, is a drug that can be very well combined with the standard of care regimens that we see, a little bit less with anti-CD38. This is why iberdomide is doing that job. But I believe that the level of efficacy we have seen and the design of the study let us believe that this drug can be better than Revlimid and pomalidomide. So the confidence of

SUCCESSOR-1 is high. And of course, it's a higher bar because it's not an add-on, is a replacement strategy, but I think the efficacy is high.

Christopher Boerner

Thanks, Cristian. And let me just also say that I'm glad to hear so many questions on CELMoDs. The CELMoD program, we're quite excited about. You're going to see exciting data at ASCO on iber and mezi. We shouldn't forget you'll also see data on golcadomide, which continues, in my view, to be a sleeper in the program just because of the high-quality data that we've seen thus far. You'll see more of that data at ASCO. Of course, behind this, we've got additional degraders, BCL6 LDD, AR LDD. And this program in this platform is quite deep, and it's nice to see these data maturing as they are across each and every one of these programs. So it's exciting to see. So stay tuned for that at ASCO and beyond.

Operator

The next question comes from Louise Chen with Scotiabank.

Louise Chen

I wanted to ask you, in addition to your ADP study for Cobenfy, I know you have several additional potential indications for Cobenfy. And which of those additional indications are you most excited by? And then secondly, you have a lot of different modalities in your cell therapy franchise and pipeline. And how do you see these all coming together to give Bristol a more comprehensive hold on the market?

Christopher Boerner

Cristian and then, Adam, you can provide color commentary.

Cristian Massacesi

So ADP, because we believe that this is a huge medical need, and we believe Cobenfy can bring benefit based on what we have seen in schizophrenia, we are focusing our strategy in psychosis, specifically assessing hallucination and delusions. And this is something that we have seen a patient really having an improvement in schizophrenia. The -- we wanted to have multiple shots on goals. This is why we have ADEPT-2 and ADEPT-4 that are similar studies with a difference of one is biomarker selected population, the other one not. And we have ADEPT-1 that is more a study that will assess how Cobenfy will avoid relapses. We also designed ADEPT-5. So we have now 4 shots on goal in this program because the base case I was mentioning before is having at least 2 positive. Things are changing maybe. And we believe this is an important setting. Cobenfy can bring benefit to patients. We want to really have multiple shots on goal. When talking -- when thinking of the other indication, bipolar disorder is the next one in line because we are expecting readout in 2027. We are testing Cobenfy specifically in mania in the context of bipolar disorders. And if you think, this is very similar in terms of the productive symptoms that we see in AD psychosis. And then we have AD agitation. AD agitation is very related to psychosis because this is one symptoms that we have seen Cobenfy already providing benefit. So the confidence is high also for this. Agitation is coming in 2028, like cognition -- AD cognition. And cognition is probably different mechanism.

Psychosis is probably mediated by muscarinic receptor 4, cognition probably by muscarinic receptor 1. But Cobenfy is working on both. So this is where the confidence is. So I would say the Cobenfy, we believe, can bring benefit in controlling these kind of symptoms in Alzheimer's, in bipolar patients and is a noncore asset for our portfolio in neuroscience. We are building -- I really would like you to start to see how we are building our portfolio in Alzheimer's with a Phase II asset and multiple Phase I assets that show the commitment that BMS has for this disease because we want to play in this disease.

Adam Lenkowsky

Yes. Just to add, Cristian, you did a great job covering much of our life cycle management program. But from a commercialization standpoint, I would say, stepping back, there are approximately 7 million patients diagnosed with Alzheimer's disease. And roughly 30% to 50% have psychosis and that's hallucination and delusions, and the vast majority have cognitive impairment. So this presents a real significant unmet need where there are no approved treatments today. And we know that antipsychotics have significant safety limitations. They have movement disorders. They carry box warnings that are specific to elderly patients with dementia. And they're often treated and used inappropriately. Rather than treating the underlying psychiatric diseases, they leave patients with cognitive impairment, falls and fractures. And these are all really serious issues in long-term care facilities. We believe that Cobenfy has potential to play a very important role in treating a number of Alzheimer's diseases. Safety becomes increasingly important in an elderly population where Cobenfy is not associated with EPS, sedation, doesn't carry a box warning. And in those 2 areas, ADP and Alzheimer's disease cognition, Cobenfy will be the first and only product approved in those.

Cristian Massacesi

Let me -- thank you, Adam. Let me go back to your second question on cell therapy. I could discuss about cell therapy strategy in hematology or in autoimmune disease. I want to focus on autoimmune disease because probably it is newer and I think is more -- it is important that we explain the strategy that we want to put in place on the development, maybe on the commercial side. I believe that BMS has a very powerful platform in this space and with one aim, a reset immune system. And to reset immune system, this is the strategy that we want to take, we decided to have a multimodal approach. This is why we have an autologous, an allogeneic and an in vivo platform. And I have to say this sets us apart compared to many other players in the space. If you think of these products, this can be transformative for patients with autoimmune disease because if you can eradicate B cells, you can provide benefit to patients with a severe or moderate state of the disease, but the vision can be to use this onetime treatment before the patients start to have the organs damaged by the autoimmune diseases. The most advanced program is Zola-cel, our autologous CAR-T. We have 2 ongoing pivotal studies, one in lupus and one in scleroderma. We have multiple other indications ongoing, and we see a level of activity that is unprecedented. And then we have in clinic now an allogeneic CAR-T that, of course, can represent a more accessible and scalable approach because from 1 donor, you can manufacture 100 cells. And so this can broaden up the access. But the real transformative thing can be in vivo. We acquired Orbital, an mRNA in vivo platform, where you have the patients that are producing, manufacturing the cells [indiscernible]. So this is really -- can be

transformative because it can really broaden up and give scalability in such a broad space like autoimmune disease. So I hope I addressed the strategy. We want to be a player. We want to lead in this space. I think we are very well set to do that.

Operator

The next question comes from Terence Flynn with Morgan Stanley.

Terence Flynn

I'll keep it to one. Cristian, I appreciate the details on the milvexian AFib trial in terms of noninferiority on the efficacy endpoint. But just was wondering if you could elaborate in terms of what differential it's powered for on bleeds for superiority? Or if you don't want to answer that question, what you think is a clinically relevant delta versus Eliquis that would drive reimbursement coverage?

Cristian Massacesi

Maybe I'll start and then, Adam, you can step in as you want. So Terence, again, the study is designed to show noninferiority. Noninferiority has margins that goes, as I was mentioning, we disclosed this margin while I can speak about it. They go from 0.83-0.84 to 1.3-something. So we believe in the study and the preclinical and clinical work done in Phase II is set to show that the noninferiority is meant to have a similar activity on efficacy versus apixaban. It is possible that milvexian can have another ratio less than 1, but it's not needed, because clinically and maybe commercially, the success required to be similar and having a better bleeding profile in major bleedings and clinically meaningful bleeding. And this is where the study, I think, is extremely well set and powered to show the noninferiority and then superiority on the bleeding [risk].

Adam Lenkowsky

Just from a coverage standpoint, Terence, what payers consistently tell us is that bleeding and particularly major bleeding is the single largest cost driver associated with oral anticoagulation therapy today. It's why Eliquis has significant share in the market. And payer discussions are suggesting that the potential of an improved benefit/risk profile will be a strong value proposition, particularly around economic benefits. And payers aren't necessarily anchored to a specific percentage threshold. What they're looking for is a clinically meaningful and statistically credible reduction in major bleeds that translates into fewer hospitalizations and fewer events that are clinically and economically important.

Operator

The next question comes from Seamus Fernandez with Guggenheim Securities.

Seamus Fernandez

Great. So if I may, just wanted to drill in a little bit on admilparant and the opportunity there. Just from a commercial perspective, we're seeing a very robust potential combination market poised to emerge here. But obviously, the key is success in clinical programs. So just wanted to get, maybe, Cristian, a little bit

more of your sense of what are the key risks as we evaluate Phase II to Phase III? And how do you see the opportunity beyond that? When we look back at the Phase II, it incorporated a Bayesian analysis from a statistical perspective. There weren't that many patients on background therapy. So just trying to get a better understanding of how you see the risk reward heading into the IPF results and the PPF results? And then just for Adam, as you look at the evolution of this market, how are you looking at the impact of the current antifibrotic standard of care and the dropout rate that patients experience and suffer from versus some of the emerging data sets for other combinations in this setting like the Treprostinil data?

Christopher Boerner

Cristian and then Adam.

Cristian Massacesi

Seamus, the -- let me start with the target. LPA1 inhibition is important because it's working in 3 dimensions in the fibrotic process in IPF and PPF. Fibrosis, inflammation and also repair. So this is the novelty of the target. And I think admilparant is the first-in-class in this space for both IPF and PPF. The goal here is to improve upon efficacy, but also to have a differentiated tolerability. You know there are drugs that the patient can use today that have some GI issues, some cough issues. So admilparant profile is very different, very differentiated. The conviction on this program is sitting on the Phase II results in both IPF and PPF. We have more than 60% improvement versus placebo in the lung function decline with 60-milligram BID. And you know we tested different doses and the dose relationship is very clear. This is another nuance very important that we put in the Phase III that give me confidence in what we are doing because we are running both studies, IPF and PPF with 2 doses, 60 and 120. The dose relationship and the benefit of having a higher dose was very clear, deeper efficacy while the dose is increasing. DMC also for these trials are continuing to monitor and reviewing the conduction of the study, safety and efficacy and tell us to continue as planned. So we do not see any flag, especially on safety side in terms of hypotension, syncope events, even overall in the trial. So 2 shots on goal with 60 and 120 in a very well-designed power studies. The Phase II and the Phase III population are very similar. We tried and the teams did a very good job in ensuring consistency and in trying to have as much as possible in Phase III what we did in Phase II. In your question on -- specifically on the design of the trial, in both trials, in IPF, we stratify based on prior treatments, pirfenidone and nintedanib or nothing. So we can use add-on in patients that don't receive any treatment. And in PPF, there is stratification based on usage or not of antifibrotic. So the studies will answer that question, and the drug can be used on top of standard of care or as a single agent. Very high confidence on how this program has been delivered on the target, execution and looking really forward to the results.

Christopher Boerner

Adam?

Adam Lenkowsky

Just quickly, Seamus. We're excited about this program as well. We believe that admilparant has the potential to play a meaningful role in both IPF and PPF with an improved efficacy and tolerability profile, as Cristian described. There remains a significant need for improved therapies that slow disease progression that are well tolerated and, ultimately, help patients manage their disease. As you were alluding to, GI tolerability remains a significant barrier where approximately 50% to 60% of patients on treatment today are discontinuing therapy by 12 months with current standard of care. And even what we're seeing with the newest approved products, the diarrhea rate is roughly 40%. What we're hearing from our thought leaders is that admilparant has the potential to be foundational as a first-line option and has the versatility of being used in combination given the expected efficacy and tolerability profile and unmet needs in the marketplace. So we've got important prelaunch activities that are underway now, and we are very much looking forward to the data readout in the second half of the year.

Charles Triano

Operator, can we please take our last question.

Operator

The last question today will come from Mohit Bansal with Wells Fargo.

Mohit Bansal

So I have a question on pumitamig. So your competitor has signaled that a VEGF/PD-1 compared to an IO or a PD-1 may be able to show a minimal regression in hazard ratio when you go from PFS to OS. We saw regression when we compare it to chemo, but with the IO combo, it may not be the same situation. How are you thinking about that given that Bristol is probably the only company with 2 IO combos on the market, and you have seen data for both LAG-3 as well as CTLA-4 on top of PD-1? So how are you thinking about this sort of regression from PD-1 to -- from PFS to OS given in the context of VEGF/PD-1? It's kind of an important investor debate right now.

Christopher Boerner

Cristian?

Cristian Massacesi

Thank you, Mohit, for the question. Let me start on the way we deliver these 2 mechanisms. I truly believe bispecifics are a better way to deliver 2 different mechanisms than using 2 different antibodies because you are much more on target, you are much more selective in delivering them and potentially, you can decrease also the off-target issues like adverse event. So there is another important learning that we have had that we are having every day with these drugs. Our pumitamig is a safe drug. The safety is very predictable. The combinability is very high. So we know VEGF is impacting PFS. This is what also you are mentioning. And I think that we will see where the data are maturing, we will see even in a few weeks at ASCO what this PFS improvement can translate in terms of OS. My base case, Mohit, is that if I have a drug that give me a good PFS gain and statistical significance OS gain that will be without increasing the safety

in a dramatic way, it is good enough because then I can use this as a backbone and I can improve even further pairing other mechanisms that can continue to increase the PFS gain and potentially translate even in a better OS. I don't think we can be so -- we can simplify so much like in the past, the VEGF inhibitors did not translate in OS because here, we have the IO component. And we still don't know how much the VEGF part of the drug is giving us the upside for the IO. And it's possible actually that this give us an uplift also for OS. So I'm excited by these bispecifics. I'm excited very much about pumitamig and the plan that we are putting in place because I truly believe that this can be the backbone for future regimens for cancer patients across indications.

Christopher Boerner

Thanks, Cristian, and thanks, everyone, for the questions. In closing, I just want to come back to where we started. We're doing what we said we would do. We're executing across the business, advancing a really differentiated pipeline that we think is going to strengthen the growth profile for the company and operating consistently with financial discipline. And of course, that discipline enables us to have flexibility to invest in growth, to pursue business development where it makes sense and, ultimately, deliver long-term value. There's always more work to do, but the foundation we've built and the momentum we're seeing gives us confidence in the trajectory of the business. So with that, thanks for joining us today. And as always, the team is available for follow-ups. Have a good rest of the day.

Operator

The conference has now concluded. Thank you for attending today's presentation. You may now disconnect.